BASELINE → TREATMENT → FOLLOW-UP

Ribociclib: checks before starting and within the early cycles

Days 1–21: ribociclib. Days 22–28: break. The endocrine partner follows its own schedule. Day 25–26 is a local operational option, not an NCCP replacement for early checks.

Check	Before first dose	Cycle 1 Day 14	Cycle 2 · start and Day 14	Cycles 3–6 · before each	Later cycles
Blood count / liver / renal	Baseline	Two-week check	Continue two-weekly checks	Before each cycle	As clinically indicated; agree the local plan
ECG · heart rhythm	Required; assess QTcF	Repeat around Day 14	If clinically indicated	If clinically indicated	If symptoms, QT concerns or interacting medicines
Electrolytes	Baseline; correct abnormalities	Additional tests if indicated	At start of cycle	At start of each cycle	As clinically indicated
Restart review	Confirm prescription and interactions	Review results; hold if required	No automatic restart	Optional local D25–26 review window	Team confirms each hold / restart plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00892 v2 · official regimen · Novartis · Kisqali product information, updated September 2026

THE WHOLE TREATMENT JOURNEY

Adjuvant ribociclib + endocrine therapy: the whole pathway

Adjuvant · HR-positive / HER2-negative early breast cancer

Ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to 3 years. The aromatase inhibitor continues daily on its own schedule; ovarian suppression is added when indicated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Local / prior treatment	Before ribociclib	Cycles 1–2	Cycles 3–6	Later / endocrine continuation
Cancer medicines NCCP 00892	Surgery ± chemotherapy ± radiotherapy as indicated	Confirm HR/HER2 and eligibility	Ribociclib 21 days on / 7 off + continuous NSAI	Repeat 28-day cycles after review	Ribociclib up to 3 years; endocrine plan may continue
Surgery EARLY	Lumpectomy / mastectomy as indicated	Wound recovery and MDT review	No routine surgery in these cycles	Rehabilitation	Report new local symptoms
Radiotherapy EARLY	Usually completed in the prior adjuvant pathway	Agree sequencing before starting ribociclib	Concurrent ribociclib / RT is not assumed	Any new RT needs oncology coordination	Endocrine may continue during RT as agreed
Blood tests NCCP 00892	Review prior toxicity	FBC, renal, liver and electrolytes	FBC / renal / liver every 2 weeks	Before each of the next 4 cycles; electrolytes at start of first 6	Then as clinically indicated; written local review schedule
Heart checks NCCP 00892 · ESC	Review QT-risk medicines	ECG: QTcF must be <450 ms to start	Repeat ECG around Cycle 1 Day 14	Additional ECG if indicated	Repeat for symptoms, QT change or interacting drugs
Cancer imaging EARLY · BONE	Confirm early-breast stage	No routine metastatic CT schedule	Assess symptoms promptly	Mammography according to follow-up plan	Annual mammography; no routine markers / CT if well

What each check is for

1

Blood tests and treatment review

Review prior toxicity → FBC, renal, liver and electrolytes → FBC / renal / liver every 2 weeks → Before each of the next 4 cycles; electrolytes at start of first 6 → Then as clinically indicated; written local review schedule

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

Sources: NCCP 00892

2

Optional pre-cycle review window

Local Day 25–26 blood / review window if adopted

Allows the team time to review results before the planned next Day 1.

Ask / act: Wait for confirmation before restarting ribociclib.

Sources: NCCP 00892

3

Heart tests

Review QT-risk medicines → ECG: QTcF must be <450 ms to start → Repeat ECG around Cycle 1 Day 14 → Additional ECG if indicated → Repeat for symptoms, QT change or interacting drugs

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

Sources: NCCP 00892 · ESC

4

Imaging and markers

Confirm early-breast stage → No routine metastatic CT schedule → Assess symptoms promptly → Mammography according to follow-up plan → Annual mammography; no routine markers / CT if well

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

Sources: EARLY · BONE

5

Bone health with endocrine treatment

At endocrine / ovarian-suppression planning; repeat by risk

Low oestrogen can affect bone strength.

Ask / act: Ask whether DEXA, fracture-risk assessment or bone protection is appropriate.

Sources: BONE · NOGG

6

Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00892

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.